No. 95-1594

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IN THE

Supreme Court of the United States

OCTOBER TERM, 1996

BARBARA A. DE BUONO, M.D., in her capacity as New York State Commissioner of Health; KAREN SCHIMKE, in her capacity as New York State Executive Deputy Commissioner of Health; DEPUTY DIRECTOR, Division of Health Care Financing, Office of Health Systems Management, New York State Department of Health,

Petitioners,

—against—

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND, by its Trustees, JOHN BOWERS, JAMES CAPO, FRANK LONARDO, WILLIAM P. LYNCH, M. BRIAN MAHER, JAMES P. MELIA, GERALD OWENS, and PETER VICKERS,

Respondents.

ON WRIT OF CERTIORARI TO THE UNITED STATES COURT OF APPEALS FOR THE SECOND CIRCUIT

REPLY BRIEF FOR PETITIONERS

BARBARA G. BILLET
Solicitor General

PETER H. SCHIFF
Deputy Solicitor General

DANIEL F. DE VITA
Assistant Attorney General
in Charge of Labor Bureau

M. PATRICIA SMITH*
Assistant Attorney General

* *Counsel of Record*

DENNIS C. VACCO
Attorney General of the
State of New York
120 Broadway
New York, New York 10271
(212) 416-8707

Attorney for Petitioners

1998

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REPLY BRIEF FOR PETITIONERS

This brief is submitted by petitioners (referred to collectively as "the State") in response to the brief filed by respondents, trustees of the NYSA-ILA Medical and Clinical Services Fund ("the Fund"), and their *amici*. In support of its argument that the Health Facility Assessment ("HFA") is pre-

empted, insofar as it is applied to Fund-owned medical centers, the Fund attempts to distinguish the application of the HFA to its medical centers from the application of the HFA to non-ERISA plan-owned hospitals, from other generally applicable taxes, and from other State regulation of the Fund's medical centers. The Fund's effort to distinguish the HFA from the surcharges at issue in *New York State Conference of Blue Cross and Blue Shield Plans v. Travelers Insurance Co.*, 514 U.S. ___, 115 S. Ct. 1671 (1995) ("*Travelers*"), and to carve out the result it desires in this case, does not withstand scrutiny. The Fund cannot distinguish the HFA from the surcharges upheld in *Travelers*. Nor can it adequately explain why even the most narrow application of its theory would not require the preemption of all state regulation where an ERISA plan opts to provide healthcare benefits in-kind.

I. GENERALLY APPLICABLE TAXES DO NOT RELATE TO AN ERISA PLAN SIMPLY BECAUSE THE PLAN PAYS THE TAXES

The Fund first argues that no extended analysis, such as that in which this Court engaged in *Travelers*, is necessary in this case because the connection between the HFA and the Fund is "clear and unmistakable." The Fund pays the tax and "unless the tax law relates, the ERISA plan would not have to pay the tax" (Resp. Br. 16).[1] This theory of ERISA preemption of state tax laws is far too broad. It would require preemption of any generally applicable tax that happens to fall on any activity in which ERISA plans engage. Sales taxes, real estate taxes, unemployment taxes and gasoline taxes all "reach into the plan's coffers to siphon off monies" (Resp. Br. 16) and would be preempted under this analysis.[2] Indeed, the

[1] References to the Fund's brief are cited as (Resp. Br. __).

[2] Under this theory no medical center income may be taxed. Yet the Fund has not challenged the imposition of the tax on non-patient care related income such as medical records fees, supplier rebates, and proceeds from the sale of scrap (Resp. Br. 7).

Fund itself eventually concedes that these types of generally applicable taxes do not in fact "relate to" plans that pay them (Resp. Br. 31).

II. A GENERALLY APPLICABLE TAX ON HOSPITAL INCOME DOES NOT RELATE TO AN ERISA PLAN WHEN PLAN PAYMENTS ARE THE SOURCE OF HOSPITAL INCOME

The Fund then proffers a narrower theory, claiming that the application of a *Travelers* methodology leads to the conclusion that the HFA relates to the Fund. The Fund attempts to distinguish this case from the surcharges upheld in *Travelers* on two grounds. First, the Fund asserts that there is a clear Congressional intent to shield ERISA plans from state taxation, as opposed to state healthcare regulation. Second, the Fund claims that the HFA directly assesses contributions and other benefit payments made by ERISA plans and plan participants and, thus, interferes with the essential purpose of the plan. The first argument ignores the statutory language of ERISA that applies the same "relates to" test to state tax laws as to any other state law. The second argument misconstrues the HFA that assesses hospital income, not plan benefits.

A. State Tax Laws Are Treated As Any Other State Law for Purposes of ERISA Preemption

The Fund's attempt to distinguish the HFA from the surcharges in *Travelers* on the basis that it is a tax is both factually and legally unsound. As a factual matter, the HFA cannot be distinguished from the surcharges in *Travelers*.[3] Both

[3] The HFA is virtually indistinguishable from the 9% assessment upheld in *Travelers*. The 9% is an assessment on aggregate hospital costs imposed upon Health Maintenance Organizations ("HMOs") and is remitted by HMOs to the State's general fund. The HFA is imposed upon "gross receipts" and is remitted by hospitals to the State's general fund. Neither the HFA nor the 9% assessment is imposed upon plans. The 9%

the 9% and 11% surcharges in *Travelers* were taxes. Like the HFA, both raised revenue for the State's general fund. In fact, this Court noted in *Travelers* that the lower courts had considered the surcharges to be taxes for purposes of the Tax Injunction Act, 28 U.S.C. § 1341. 115 S. Ct. 1671, 1675 n. 4. Like the HFA, the surcharges also had healthcare related purposes.[4] Moreover, taxes, like healthcare regulation, are a traditional exercise of state authority. Indeed, they are the "lifeblood of government." *Franchise Tax Board of Cal. v. U.S. Postal Service*, 467 U.S. 512, 523 (1984) (quoting *Bull v. United States*, 295 U.S. 247, 259 (1935)).

The Fund's reliance on the HFA's status as a tax is also legally unsound. The standard for ERISA preemption of state tax laws is no different from the standard for ERISA preemption of any other state law. Tax laws are preempted to the extent they "relate to" plans. The Fund acknowledges this (Resp. Br. 25-26). The Fund's reliance upon section 514(b)(5)(B)(i) of ERISA, as indicating some special Congressional concern that state tax laws be preempted, is misplaced. This section provides that "[n]othing in subparagraph (A) shall be construed to exempt from subsection (a) . . . any State tax law relating to employee benefit plans." 29 U.S.C. § 1144(b)(5)(B)(i). Subparagraph A provides an exemption from ERISA preemption to the Hawaii PrePaid Health Care Act. Thus, section 514 (b)(5)(B)(i) is only an exception to the exemption from ERISA preemption accorded to the Hawaii act in section 514 (b)(5)(A), 29 U.S.C. § 1144 (b)(5)(A).

assessment is imposed upon HMOs and the HFA is imposed upon hospitals. The effect of both assessments upon ERISA plans is identical. The HFA, like the 9% assessment, increases the cost of providing healthcare benefits.

[4] The 9% assessment was imposed upon HMOs to encourage them to enroll medicaid recipients in an attempt to decrease medicaid costs. The 11% was imposed upon the hospital bills of commercial insureds in an effort to support Blue Cross's open enrollment policy. *See Travelers*, 115 S. Ct. at 1674.

Moreover, by using the same "relates to" language, Congress indicated its clear intent that state tax laws be treated like any other state law for purposes of ERISA preemption.[5]

The hospital assessment at issue here differs significantly from the laws involved in the tax cases relied upon by the Fund. Unlike the tax at issue here, in those cases state taxes or assessments that were directed specifically at ERISA plans were held to be preempted. *E-Systems, Inc. v. Pogue*, 929 F.2d 1100 (5th Cir.), *cert. denied*, 502 U.S. 981 (1991) (state law that expressly imposed a 2.5% tax on claims and benefits paid under plans and on plan administrators' fees is preempted); *General Motors Corp. v. California Bd. of Equalization*, 815 F.2d 1305 (9th Cir. 1987), *cert. denied*, 485 U.S. 941 (1988) (tax computed on the basis of benefits paid by self-insured plans relates to plans because reference to plan activities is unavoidable); *Birdsong v. Olson*, 708 F. Supp. 793, 798 (W.D. Tex. 1989), *appeal dismissed*, 901 F.2d 1270 (5th Cir. 1990) (the same law as was at issue in *E-Systems* is preempted); *National Carriers' Conference Comm. v. Heffernan*, 454 F. Supp. 914 (D. Conn. 1978) (law that required ERISA plans to pay a tax on amounts paid as benefits is preempted because it is specifically directed at plans). These cases fit within the Court's holdings which have preempted state laws which refer to, are predicated upon, or which function only through the existence of, ERISA plans. *District of Columbia v. Greater Washington Bd. of Trade*, 506 U.S. 125 (1992); *Ingersoll-Rand Co. v. McClendon*, 498 U.S. 133 (1990).

State tax laws of general applicability, like the tax at issue here, which happen to catch employee benefit plans within their wide sweep, have not been preempted. *See Thiokol Corp., Morton International, Inc. v. Roberts*, 76 F.3d 751,

[5] The legislative history of this section is recounted in detail in *Retirement Fund Trust of Plumbing v. Franchise Tax Board*, 909 F.2d 1266 (9th Cir. 1990), which concluded that the same "relates to" test of preemption that applies to state laws generally applies also to state tax laws.

753-57 (6th Cir. 1996), *petition for cert. filed*, 64 U.S.L.W. 3008 (U.S. May 23, 1996) (No. 95-1913) (tax which requires businesses to include within their tax base all compensation, including payments to ERISA plans, is not preempted by ERISA); *Firestone Tire & Rubber Co. v. Neusser*, 810 F.2d 550, 554 (6th Cir. 1987) (municipal ordinance which imposes a 2% tax on all income earned by all residents, including ERISA plan contributions, is not preempted. Despite the argument that it had a direct effect on the funding of ERISA plans, the ordinance was a neutral tax of general application taxing all income regardless of its ultimate disposition).

The Fund mischaracterizes the State's argument concerning laws of general applicability. The State does not claim that all laws of general applicability are immune from ERISA preemption. Rather, the State acknowledges that laws of general applicability that dictate or restrict plan administration or benefit structure may relate to plans. Laws that are specifically directed at employee benefit plans, *Mackey v. Lanier Collection Agency and Serv., Inc.*, 486 U.S. 825, 830 (1988), or that depend upon the existence of employee benefit plans, *Ingersoll-Rand*, 498 U.S. 133, also relate to plans.

Nor, contrary to the Fund's argument (Resp. Br. 20), does the State claim that laws with only an economic impact on plans are immune from preemption. A law with only an economic impact on plans nevertheless "relates to" plans when it targets ERISA plans, *Greater Washington Bd. of Trade*, 506 U.S. 128; *Mackey*, 486 U.S. 830, when it depends upon the existence of ERISA plans, *Ingersoll-Rand*, 498 U.S. 133, or when its economic effect upon a plan is so "acute" that it effectively "forces an ERISA plan to adopt a certain scheme of substantive coverage or effectively restricts its choice of insurers," *Travelers*, 115 S. Ct. at 1683.

Many of the Fund's arguments in support of preemption of the HFA under a *Travelers* analysis are, in fact, identical to arguments rejected by this Court in *Travelers*. Thus, the arguments that the HFA relates to the Fund because its economic impact will deplete plan assets (Resp. Br. 22), will implicate interstate uniformity by varying the cost of benefits from state to state (Resp. Br. 21), and will provide an economic incentive to restructure administrative proceedings (Resp. Br. 21), have already been rejected by this Court. *See* 115 U.S. at 1679-80.

The Fund's argument that the application of the HFA to its medical centers is in violation of section 302 of the Labor Management Relations Act of 1947, 29 U.S.C. § 186, and section 403(c)(1) of ERISA, 29 U.S.C. § 1103(c)(1), commonly known as the exclusive benefit rule, is unavailing. The argument would apply equally to all taxes. The suggestion that the payment of healthcare taxes is in violation of the exclusive benefit rule has been rejected by every Court of Appeals that has addressed the issue. *See Boyle v. Anderson*, 68 F.3d 1094 (8th Cir. 1995), *cert. denied*, ___ U.S. ___, 116 S. Ct. 1226 (1996); *New England Health Care Employees Union, District 1199 v. Mount Sinai Hospital*, 65 F.3d 1024, 1032 (2d Cir. 1995); *United Wire, Metal & Machine Health & Welfare Fund v. Morristown Memorial Hosp.*, 996 F.2d 1179, 1196 (3d Cir.), *cert. denied*, 510 U.S. 944 (1993).

In the end, the Fund's argument that the HFA relates to plans because it is a tax begs the question. The Fund's claim that the HFA relates to plans, and is distinguishable from the surcharges in *Travelers*, is squarely based upon the fact that the HFA is a tax, and not on a claim that the HFA affects plans differently from the surcharges upheld in *Travelers*. Taxes are preempted only if they "relate to" plans. Thus, the mere fact that a law imposes a tax on plans cannot alone establish the claim that the law "relates to" plans.

B. The HFA Assesses Hospital Income, Not ERISA Plan Benefits

The Fund also argues that the HFA relates to ERISA plans, and is distinguishable from the surcharges upheld in *Travelers*, because it directly taxes contributions and other payments for health care benefits. This characterization misconstrues the HFA and its effect on plans. The HFA is not an assessment imposed upon employer contributions. Nor is it a tax on benefits paid by ERISA plans. It is a tax on hospital income. Despite the Fund's characterization, it is unable to articulate a coherent theory under which the HFA would be preempted as applied to patient care income received by its medical centers, but not preempted as applied to patient care income that any non-fund owned hospital can trace to an ERISA plan.

The HFA is an assessment imposed upon hospitals measured by hospital receipts "for or on account of hospital and health related services," regardless of source. It was not designed to tax ERISA plans or ERISA benefit payments. Rather, the HFA covers the Fund's medical centers only because they come within the wide sweep of the covered class of "hospitals." Payments to the medical centers for patient care services are subject to the assessment only because they fall within the broad definition of "gross receipts." A payment to a hospital from an ERISA plan may constitute a benefit to the plan participant on whose behalf the payment is made. To the ERISA plan that pays it, the payment may constitute the payment of benefits. But to the hospital, the taxed entity, it is simply income.

The laws at issue in *Travelers* set hospital rates for all patients, imposed surcharges on certain rates, and imposed an assessment on HMOs. N.Y. Pub. Health Law §§ 2807-c(1)(a), (b), (c), 2807-c(2-a). Their effect on ERISA plans is identical to that of the HFA. First, none of these laws applies to the plans themselves. Second, when ERISA plans provide hospital care benefits to their participants, the HFA, like the laws

at issue in *Travelers*, may increase the cost of providing such benefits to plans.

This analysis is not affected by the fact that an ERISA plan may own and operate a medical center subject to the assessment. If the HFA can be characterized as a tax "on contributions and other payments for healthcare benefits provided by ERISA plans" (Resp. Br. 10), that characterization is applicable anytime any ERISA plan pays money to any hospital on behalf of a plan participant. Ownership of the hospital is irrelevant to this characterization. The Fund itself does not differentiate between income the medical centers receive from employers who participate in its own plan and patient care income from other plans. Instead, the Fund's position is that ERISA preempts the HFA even when the assessed income can be traced to ERISA plans unrelated to the Fund (Resp. Br. 7,8). Thus, according to the Fund, payments from the PPGU fund, which is unrelated to the NYSA fund, are preempted from being included in assessable income. The Fund also argues that payments from workers' compensation carriers, who are also third parties to the Fund, cannot be included in assessable income. Yet, with regard to these entities, the Fund's medical centers operate no differently than any other non-fund owned hospital. They provide medical services for which they receive payments that constitute income to the centers.

All hospitals receive a large portion of their income for patient care services from ERISA plans. The only difference between this case and a case involving a hospital not owned by an ERISA plan is the portion of the hospital's income that is derived from ERISA plan payments. In this case, all assessable income from "patient care services" comes from ERISA plans, either the PPGU plan, the NYSA plan, or workers' compensation carriers. In the case of a hospital open to the general public, income will come from medicaid payments, medicare payments, self-insured ERISA plans, insured ERISA plans, non-ERISA employee benefit plans, private insurance

and individuals' payments. However, since 88% of non-elderly Americans get their health insurance through their employer,[6] a very large portion of any hospital's income could be characterized as payments and contributions which are intended to pay for ERISA plan participants' medical benefits.

Moreover, the effect of the HFA on the ERISA plans which pay for medical benefits is the same whether or not a hospital is owned by an ERISA plan. The PPGU plan, which makes payments to the Fund's medical centers, the NYSA plan employers, which send contributions to the medical centers, as well as all the ERISA plans that make payments to non-fund owned hospitals, are economically affected by the HFA.

III. A GENERALLY APPLICABLE TAX ON HOSPITAL INCOME DOES NOT RELATE TO AN ERISA PLAN SIMPLY BECAUSE THE PLAN OWNS THE HOSPITAL

Relying on section 3(1) of ERISA, 29 U.S.C. § 1002(1), which defines a plan as an entity whose purpose is to provide healthcare benefits "through the purchase of insurance or otherwise," the Fund argues that its ownership of the medical centers distinguishes this case from *Travelers*. According to the Fund, "[i]n dispensing care and treatment at its clinics the Fund is engaging in the very activity" that defines it as an ERISA plan (Resp. Br. 24). Thus, the Fund argues, it does not take "Jamesian metaphysics" to see that *all* state regulation of healthcare relates to the plan when an ERISA plan provides healthcare benefits in-kind (Resp. Br. 31-32). This theory of ERISA preemption would free ERISA plans from state regulation whenever they opted to provide any benefit in-kind. Nor, under this theory, can the Fund distinguish the HFA from

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[6] *See The Travelers Insurance Co. v. Cuomo*, 14 F.3d 708, 711 (2d Cir. 1994), *rev'd sub nom. New York State Conference of Blue Cross and Blue Shield Plans v. Travelers Insurance Co.*, 514 U.S. ___, 115 S. Ct. 1671 (1995).

other generally applicable taxes such as unemployment taxes imposed upon its employment of doctors, or sales taxes imposed upon its purchase of medical supplies.

ERISA defines a plan as a program whose purpose is to provide certain benefits. By definition, benefits are separate and distinct from plans. A benefit is not magically transformed into a plan when a plan chooses to provide it in-kind. For example, when plans provide benefits "through the purchase of insurance," not all operations of the insurance company become part of the plan. *Metropolitan Life Ins. Co. v. Massachusetts*, 471 U.S. 724, 741 (1985). When plans provide benefits "otherwise" through HMOs, which also provide medical care in-kind, not every aspect of the provision of medical care implicates ERISA preemption. *See Dukes v. U.S. Healthcare, Inc.*, 57 F.3d 350 (3d Cir. 1995).

In *Travelers*, the HMOs made arguments concerning their relationship to ERISA plans quite similar to the Fund's argument in this case. There the HMOs relied upon the statute's "otherwise" language to argue that when a plan contracts with an HMO which provides medical care, the HMO is the mechanism through which the plan "otherwise" provides benefits. The HMO, therefore, is an integral part of the plan. The HMOs argued that they were immune from state regulation because there is a "confluence between the employee benefit plan and the HMO which is the entity that is actually providing for [the] participants . . . the medical, surgical or hospital care set forth in the statute." Brief for Respondents New York State Health Maintenance Organization Conference *et al.* at 20, submitted in *New York State Conference of Blue Cross and Blue Shield Plans v. The Travelers Insurance Co.*, 514 U.S. ___, 115 S. Ct. 1671 (1995) (Nos. 93-1408, 93-1414, 93-1415). This Court rejected these arguments *sub-silentio*.

The "relate to" standard urged by the Fund, which would preempt all laws that affect a plan's provision of benefits, provides no limitation on the scope of ERISA preemption.

Employee benefit plans may not engage in activities unrelated to the provision of benefits. They cannot, for example, operate amusement parks or shoe stores since ERISA requires all assets be used for the exclusive purpose of providing benefits to participants or deferring reasonable administrative expenses. 29 U.S.C. § 1103(c)(1). This is true whether or not a plan provides benefits in-kind. Thus, under the Fund's theory, all plan activities are shielded from state regulation. Further, while the Fund admits that it is subject to state law when it is engaging in activities that "are not peculiar to employee benefit plans but are carried on by other private entities" (Resp. Br. 31), when a plan does provide healthcare, day care, or prepaid legal services in-kind, it is in fact engaging in activities in which many other private entities engage. This contradiction renders the Fund's proposed standard meaningless.

In order to provide benefits through its medical centers, the Fund must hire employees, including nurses, doctors and medical records clerks. It must purchase medical supplies and pay utility bills for necessary electricity. All of these functions are essential to the operation of a hospital and, therefore, under the Fund's theory, affect the plan as a plan (Resp. Br. 31). Yet, the Fund concedes that when a plan operates as an employer or a purchaser of goods and services it may be regulated by the State. State unemployment taxes on plan doctors, sales taxes on syringes, gasoline taxes on plan-owned ambulances, and minimum wage requirements for medical records clerks affect the provision of benefits to the same extent as staffing requirements and medical waste regulations. Once again, the Fund's test casts too wide a net and is unable to distinguish functions which are peculiar to employee benefit plans from those in which many other entities engage.

Perhaps realizing that a test for ERISA preemption that invalidates all state laws that affect a plan's in-kind provision

of benefits is untenably broad,[7] the Fund finally retreats from this position. Instead, the Fund offers a new test of preemption: although all regulation "relates to" the in-kind provision of benefits provided by ERISA plans, only those laws that do not involve the historic police powers of the state are preempted (Resp. Br. 32). The Fund places tax laws outside this arena, but refuses to speculate as to where other state laws would fall. The Fund frankly concedes that such a test requires a case-by-case determination (Resp. Br. 32). This test ignores the statutory language of ERISA, which treats tax laws like all other state laws. Instead, it substitutes an inquiry tailored to the facts of this case for statutory analysis.

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[7] Such a test would displace all state regulation anytime a plan opted to provide healthcare, day care or legal services benefits in-kind. Thus, despite the Fund's attempt to make light of the State's concern about "unqualified nurses, unprotected children, unethical lawyers and unembalmed bodies" (Resp. Br. 30), it is a real concern under the Fund's theory.

CONCLUSION

For the reasons set forth here and in the State's main brief, the Court should reverse the judgment below.

Dated: New York, New York
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Respectfully submitted,

DENNIS C. VACCO
Attorney General of the
State of New York
120 Broadway
New York, New York 10271
(212) 417-8707

BARBARA G. BILLET
Solicitor General

PETER H. SCHIFF
Deputy Solicitor General

DANIEL F. DE VITA
Assistant Attorney General
in Charge of Labor Bureau

M. PATRICIA SMITH*
Assistant Attorney General
of Counsel

* *Counsel of Record*